

Emergency Decision Unit Care plan

Last hours of Life

Date: DD/MM/YY

Patient details

Full name

DoB

Unit number

(use sticker if available)

Admission Checklist

Patient recognised as dying with estimated prognosis of <24 hours – Preferred place of death is hospital

Patient has been assessed by doctor (ST4 or above) and recognised as dying, is not for CPR and condition assessed to be non-reversible. A ReSPECT form has been completed and includes these decisions.

The patient's case has been discussed with EPIC who agrees that the patient is appropriate for end of life care on EDU

Relatives (and patient if capacity to be involved in discussion) are fully updated of the patient's condition, understand the patient is dying and the plan of care is agreed.

If felt appropriate, ask about preferred place of death. Where this is not hospital, refer to the Emergency Floor Discharge Practitioners and consider if moving to EDU is still appropriate.

Patient has been e-referral to EDU and accepted by them, taking into account bed capacity of the unit and clinical dependencies. Nurse capacity/ skill. And prioritized where appropriate for bed space.

If there is uncertainty about plan of care, or advice needed on complex symptom control please refer to the SPCT Nervecentre or ICE. If the referral is urgent – please call the team to discuss - 07814364595 (0900-1700 Mon-Sun). Outside of these hours, advice can be sought via switchboard from the on call Consultant in Palliative Care. For more information about the SPCT & referral criteria visit - [UHL Specialist Palliative Care Team](#) | [CHUGGS - UHL Connect](#)

Electronic medicines chart reviewed and anticipatory medications for end of life must be prescribed (prescribing bundle on Nerve Centre – Follow guidance for renal dosing)

EDU plan of care

- **EDU nursing to initiate or continue SWAN 4 hourly nursing assessment on Nervecentre**
- If there is uncertainty about plan of care, or advice needed on complex symptom control please refer to the SPCT Nervecentre or ICE. If the referral is urgent – please call the team to discuss - 07814364595 (0900-1700 Mon-Sun).
- Some patients attending the ED are in their last hours/days of life. Although very difficult to determine prognosis, patients with a very short expected time to live can be cared for on EDU. If prognosis changes or timeframes become extended discussion about bed placement or place of care to be reviewed by EDU COTD

The [Symptom control in last days of life \(including diabetes management\)](#) guidance is a very useful guide to ensure quality care of dying patients. [The SWAN care plan](#) should be completed to support individualised care. Please visit [Dashboard | End of Life - UHL Connect](#) for links to policies and more information about support available for dying patients and their families.

Planned & agreed by

Referring Clinician

Print name

NB: Pathway must be completed and signed by the referring clinician. Once agreed, the EPIC or an ED consultant must record 'EDU admission on anaphylaxis approved' in the 'ED Flow Notes' box on Nervecentre.

Patients will NOT be accepted to EDU until this has been done

Signature